

Cognitive Distortions — Clinical Quick Reference

CBT · Clinician & Client Reference Card · Based on Beck, Burns & Clark (2004)

Cognitive distortions are systematic errors in thinking that maintain psychological distress. They are not moral failings — they are learned cognitive habits. Use this reference to identify, name, and gently challenge distorted thinking with your clients.

All-or-Nothing Thinking (Black-and-White Thinking)

Viewing experiences in absolute, extreme categories with no middle ground.

"If I'm not perfect, I'm a total failure."

Challenge: Ask: Is there a middle ground? What would a partial success look like?

Catastrophizing (Fortune-Telling: Negative)

Predicting the future negatively, or exaggerating the importance of problems.

"I'll completely fall apart during the interview and they'll think I'm an idiot."

Challenge: Ask: What is the worst realistic outcome? How likely is it? Could you cope?

Mind Reading

Assuming you know what others are thinking, without checking, usually negatively.

"She didn't say hello — she must be angry at me."

Challenge: Ask: What is the evidence? Could there be another explanation?

Emotional Reasoning

Assuming that negative feelings reflect reality: 'I feel it, therefore it must be true.'

"I feel like a fraud, so I must be one."

Challenge: Ask: Is this a fact or a feeling? Can feelings be inaccurate? What are the facts?

Overgeneralization

Drawing sweeping, global conclusions from a single or few negative events.

"I failed this exam. I'm terrible at everything. I'll never succeed."

Challenge: Ask: Is this always true? Are there exceptions? What does one event actually prove?

Personalization

Blaming yourself for things that are not entirely your responsibility.

"My student is struggling — it's all my fault as their teacher."

Challenge: Ask: What other factors contributed? What was your actual responsibility?

Should Statements (Musturbation)

Applying rigid rules ('should,' 'must,' 'have to') to self or others.

"I should be able to handle this. I shouldn't need help."

Challenge: Ask: Says who? What is the rule based on? What happens if you don't?

Mental Filter (Selective Abstraction)

Focusing on a single negative detail while ignoring the whole picture.

"I got one critical comment in a 20-item evaluation. The whole thing was bad."

Challenge: Ask: What is the complete picture? What are you filtering out?

Discounting the Positive

Rejecting or minimizing positive experiences: 'That doesn't count.'

"They complimented my work, but they were just being nice."

Challenge: Ask: What would it mean to simply accept the positive as valid? Why doesn't it count?

Jumping to Conclusions

Making negative interpretations without adequate supporting facts.

"She hasn't replied. She's definitely upset with me."

Challenge: Ask: What are the actual facts? What are alternative explanations?

How to Use This in Session

Clinical Note: When identifying distortions with clients, use collaborative language: 'I notice your mind might be doing something called all-or-nothing thinking — does that fit?' Never apply distortion labels pejoratively. The goal is to build the client's metacognitive awareness, not to prove their thinking wrong.